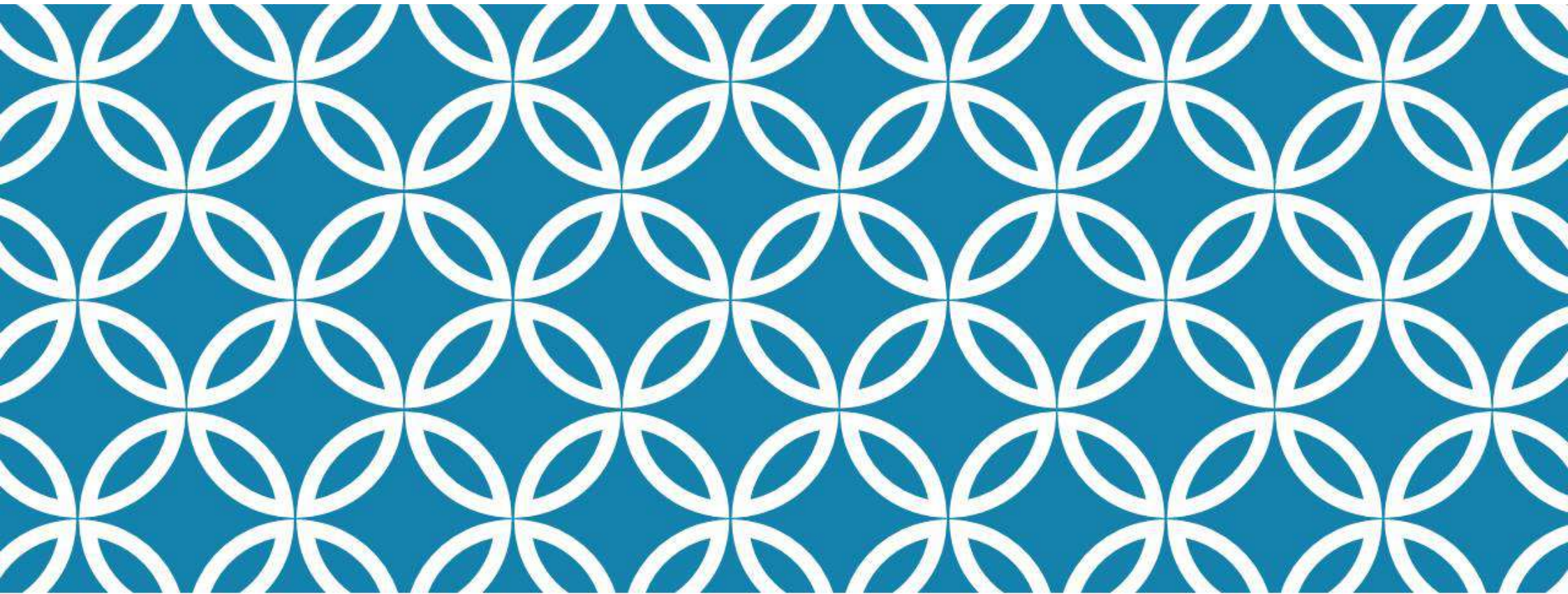




OTHER LICHENOID DERMATOSIS

ERYTHRODERMA

Dr eman gomaa
Consultant dermatology

LICHEN NITIDUS CD4



- asymptomatic multiple tiny discrete dome shaped skin colored papules
- uncommon, Children, young adult, more in female
- Dorsal aspect of hand, upper extremities. Flexors, genitalia
- Variants:
 - palmoplantar,
 - linear, follicular,
 - generalized
 - perforating



- **Oral** : rare
- **Nail** :(10% of patients (adult).
pitting, longitudinal ridging
- **Aeteopathogenesis:**
 - Unkown
 - May be unusual variant of lp
 - TB
- **Association :DMC:**
 - Down syndrome
 - Megacolon
 - Crohns disease

DD

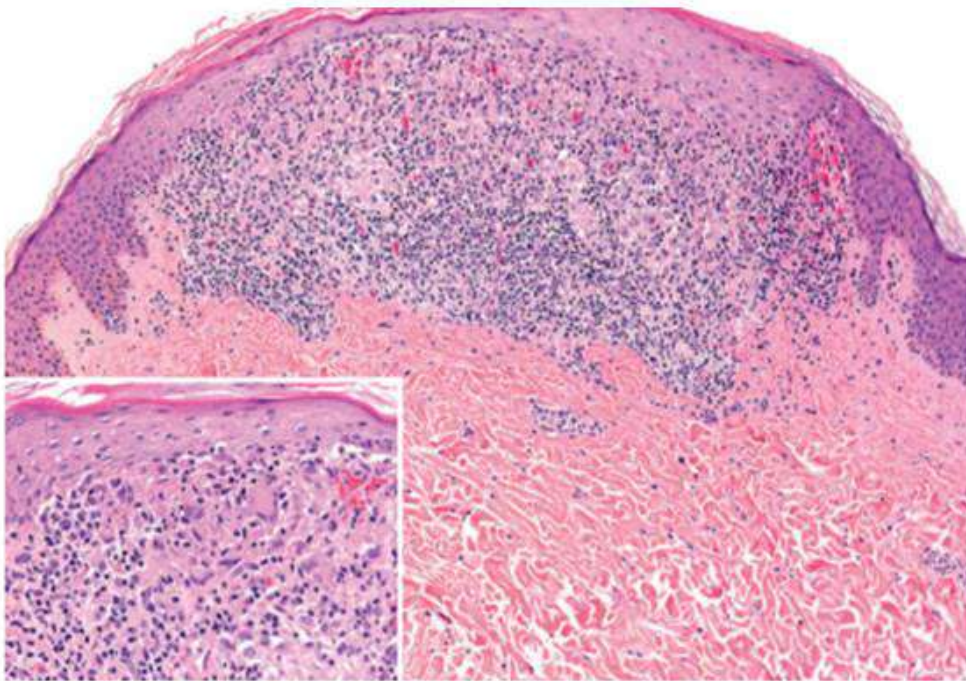


Plan wart



Friction lichenoid dermatitis

CONNECT ACADEMY - DERMA - BY DR. EMAN



➤ **H.P:(ball , clow)**

- (well circumscribed dermal infiltrate of epithelioid histiocytes cells, lymphocytes (granuloma)
- Lichenoid interface dermatitis
- Epidermal atrophy.

➤ **Treatment**

No tt > asymptomatic ,
Spontaneous clearing
Topical steroid ,
antihistaminic

COMPARISON OF CLINICAL AND HISTOLOGIC FEATURES OF LICHEN
NITIDUS VERSUS LICHEN PLANUS

	Lichen nitidus	Lichen planus
<i>Clinical</i>		
Size	Pinhead	Pinhead to larger plaques
Color	Skin-colored	Violaceous
Number	Multiple to numerous	Multiple
Distribution	Upper extremities (flexor aspects), genitalia, chest, abdomen	Wrists, flexor forearms, presacral area, dorsal hands, shins, genitalia
Oral involvement	Rare*	Common (>50%)
Nail involvement	<5–10%	~10%
Wickham striae	Absent	Common
Hyperpigmentation	Uncommon	Common
Hypopigmentation	Common†	Unusual
Koebner phenomenon	Common	Common
Pruritus	Sometimes	Common
<i>Histologic</i>		
Parakeratotic cap	Common	Rare
Granular layer	Thin to absent	Focally increased
Dermal infiltrate	Lymphocytes, epithelioid cells, and occasional Langhans giant cells; usually confined to width of 2–3 dermal papillae; "ball and claw" configuration due to hyperplastic rete ridges	Band-like infiltrate of lymphocytes
Transepidermal elimination	Unusual	Rare
Deposits of immunoreactants	Absent	Globular deposits of IgG, IgA, IgM, and/or C3 in the papillary dermis
*May be underestimated.		
†Especially in darkly pigmented individuals.		

LICHEN STREATUS



LICHEN STREATUS



- uncommon , Asymptomatic , self limited .
- C/P :
 - asymptomatic streak of pink lichenoid papules
 - unilateral , extremity along blasenko line
- Children , more in femal ,
- Seasonal variation: Summer , spring



➤ Appear suddenly , spontaneouse resolution with in months to years.

➤ Aetiopathogensise:

➤ Unkown

➤ Somatic mosaicisim

➤ Viral

➤ Cell mediated cd8

➤ Atopy ,

➤ trauma

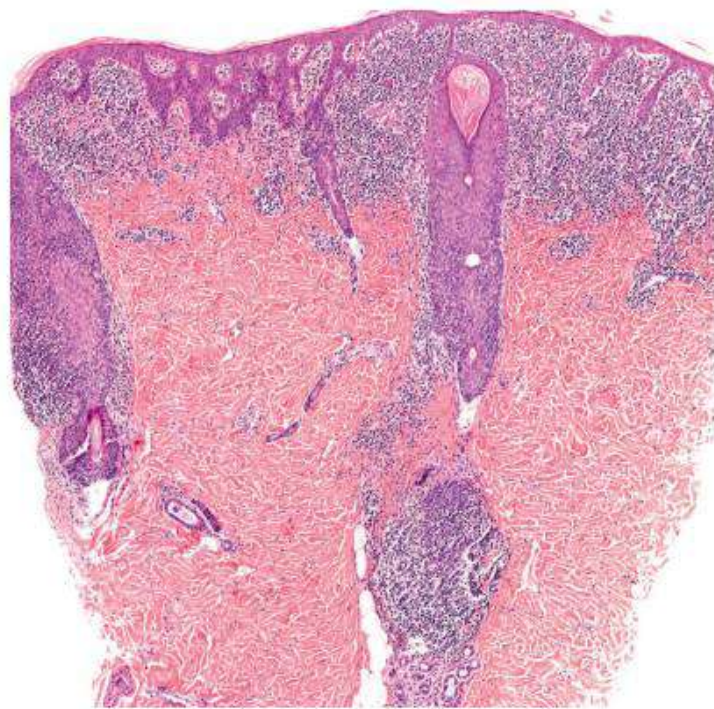


Fig. 11.20 Lichen striatus – histopathologic features. In addition to hyperkeratosis with focal parakeratosis, both a lichenoid and a perivascular and periadnexal lymphocytic infiltrate extending into the deeper dermis is seen.
Courtesy, Lorenzo Cerroni, MD.

➤ H.P:

- eczema like : spongiosis, exocytosis
- Vacular interface dermatitis
- infiltrate around hair follicle , eccrine sweat gland

DD:

- linear lp (older age, pruritic,, healing with hyper-pigmentation)

linear GVHD

lashkitis

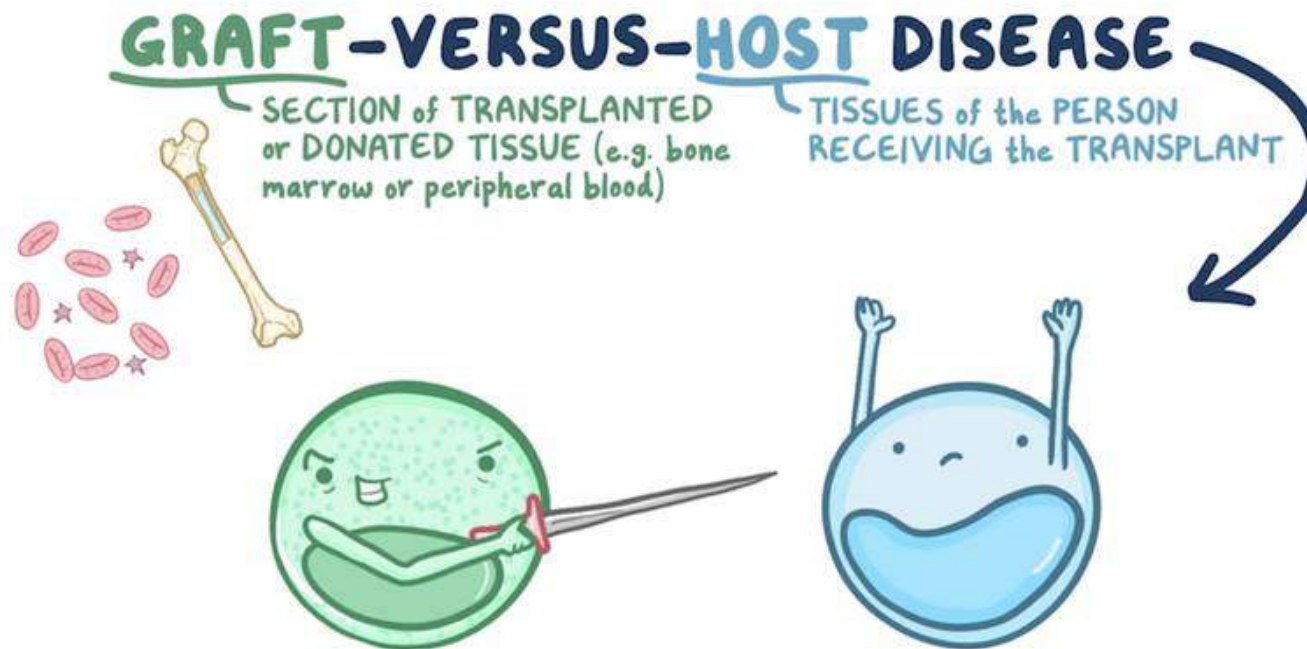
linear porokeratosis

ilven

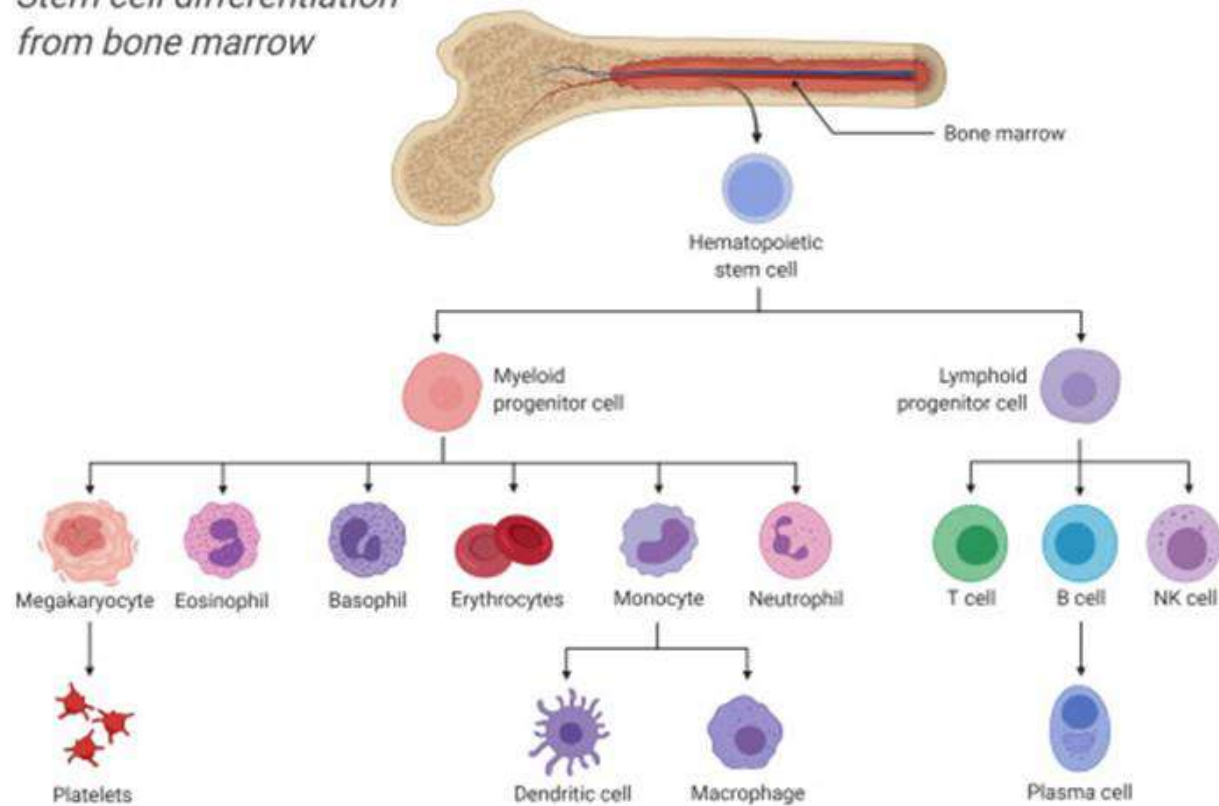
Treatment:

Resolve within 6 months

GRAFT VERSUS HOST DISEASE



*Stem cell differentiation
from bone marrow*



What to Know About Hematopoietic Stem Cell Transplantation

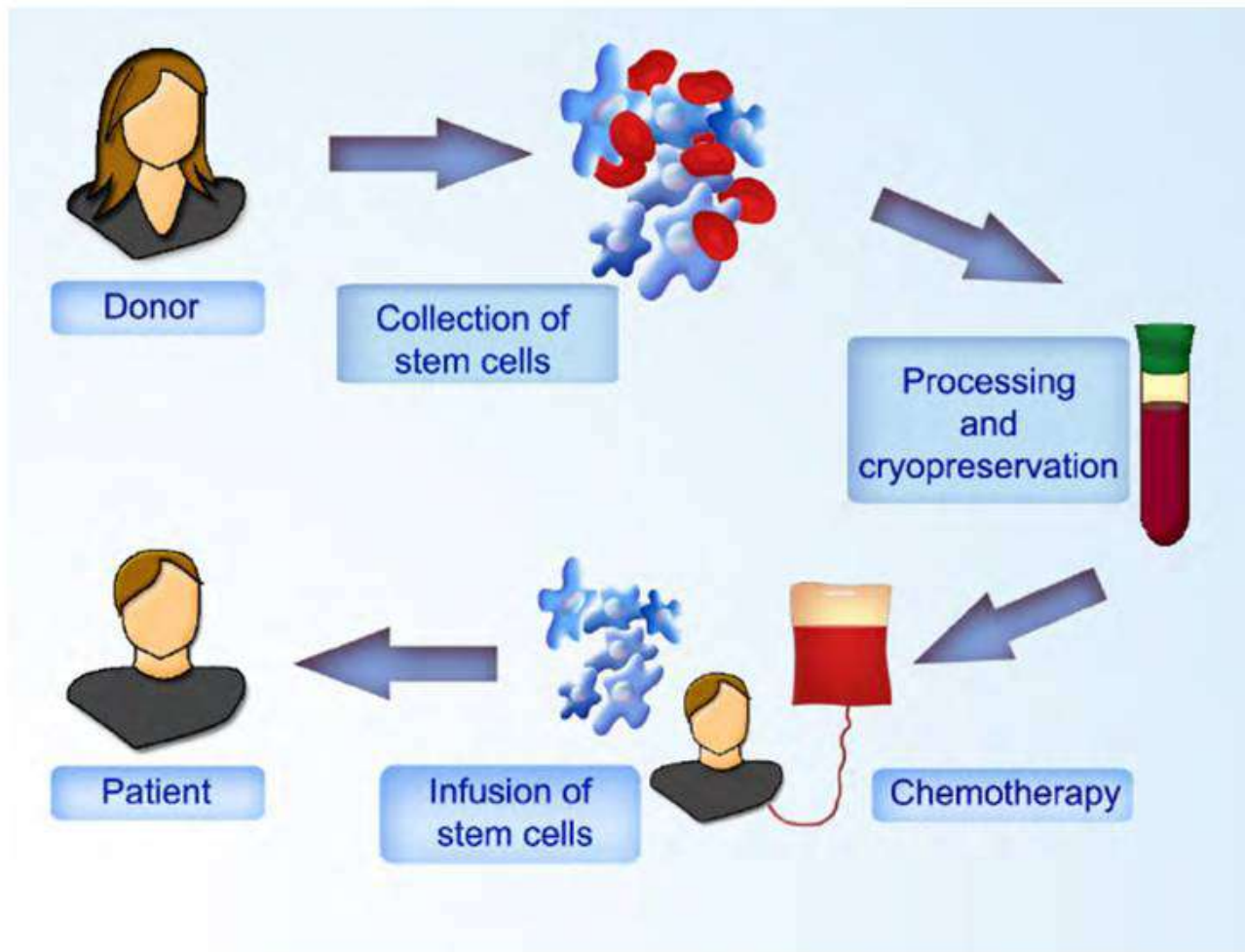


Treats some blood cancers and immune disorders

A conditioning regimen is required (which can include radiation and/or chemotherapy medications)

Donors can be the recipient themselves or a donor with a matching HLA type may volunteer

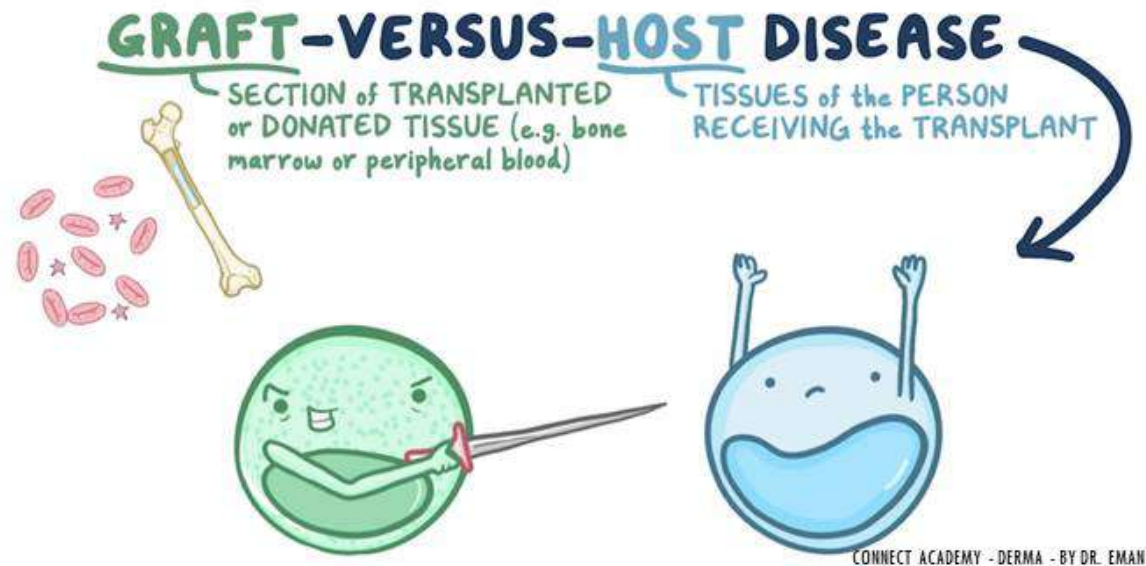
Infusion may take several hours



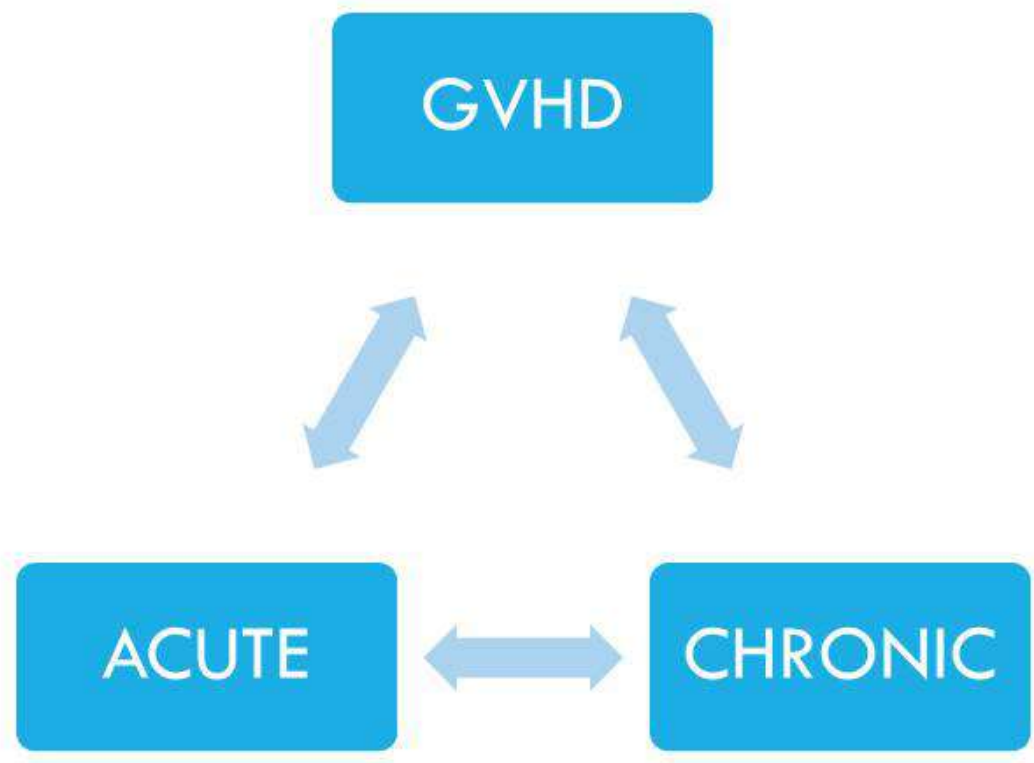
GRAFT VERSUS HOST DISEASE

DEFENITION :

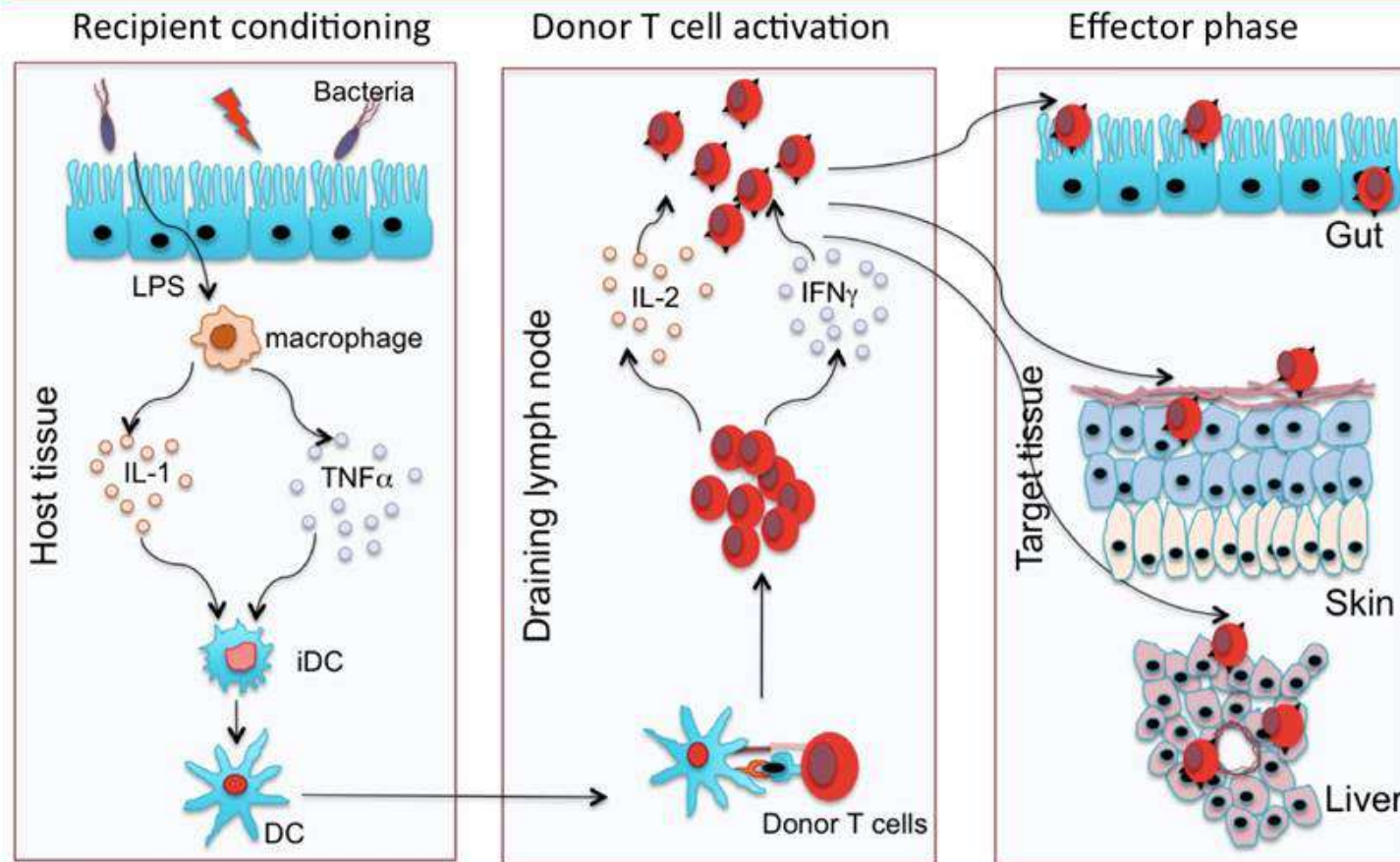
Multiorgan disease caused by transfer forign donor hematopoitic stem cell into host recipient (allogeneic HSCT).



|



Patho-physiology of GVHD



Phase 1
acute inflammation
& tissue injury

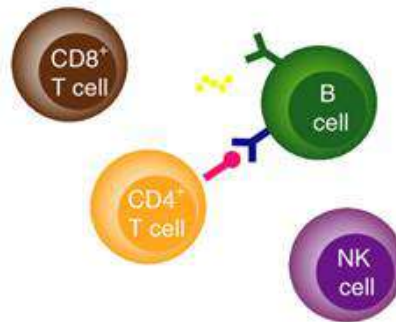
Innate immunity



- Cytokines (steroids, etanercept, tocilizumab)
- TLR-agonists
- Neutrophils (steroids, montelukast)
- Platelets
- Vascular inflammation

Phase 2
chronic inflammation
& dysregulated immunity

Adaptive immunity



- Thymic injury & dysfunction
- T cells (CNI, mTOR-Inh., MMF, ruxolitinib, baricitinib, cyclo., pentostatin, MTX, abatacept, alefacept, TNF)
- B cells (ibrutinib, MMF, rituximab, tocilizumab)
- Plasmablasts (proteasome inhibitors, imids)
- Antigen presenting cells (steroids, mTOR-inhibitors)
- Treg (IL-2, ECP, MSC) & B reg cells

Phase 3
aberrant tissue repair
& fibrosis

Innate & adaptive



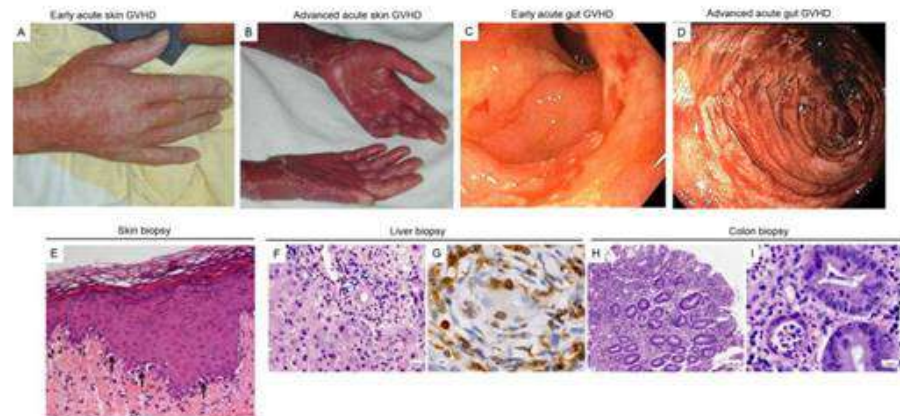
- TGFβ (pirfenidon, nintedanib)
- PDGFα (imatinib, nilotinib)
- Fibroblasts (lenabasum)
- TNF (etanercept)
- IL17 (belumosudil)
- Macrophages (axatilimab)

Legend: mTOR-Inh: mTOR-inhibitors (sirolimus; everolimus); imids: thalidomide, pomalidomide, proteasome-inhibitors: bortezomib, ixazomib; cyclo: cyclophosphamide, listing of one target dose not exclude additional targets in specific drugs (i.e.: proteasome inhibitors target T & B cells and plasma cells)

ACUTE GVHD

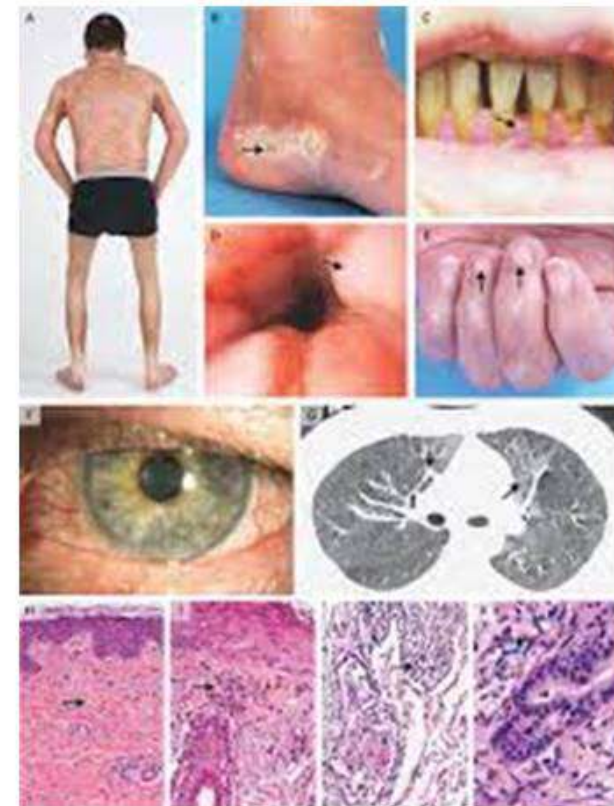
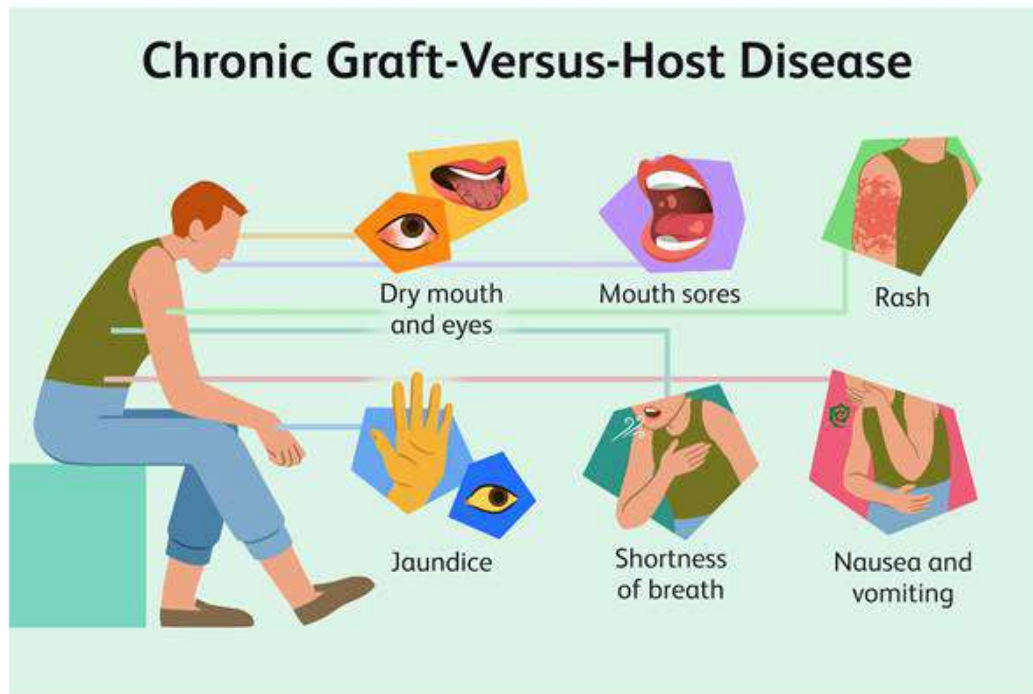


Actas Dermosifiliogr. 2016;107:183-93



CHRONIC GVHD

Chronic Graft-Versus-Host Disease



	ACUTE GVHD	CHRONIC GVHD
PATHOGENESIS	3 PHASES Recipient tissue damage Activated donor lymphocytes Alloreactive donor Tcell > cytotoxicity > tissue injury (skin, liver, GIT)	1. Roll of allo reactive t cell 2. B cell activation autoantibodies ANA, ANT dsDNA, (any body system)
C/P	Morbiliform exanthem (erythematous maculopapular rash) > erythroderma, bullae, desquamation acral areolae generalized pruritis, cicatricial alopecia GIT: nausea, vomiting, diarrhea, abdominal pain. Liver: hepatitis	LP, LSEA, morphea, scleroderma, poikiloderma, eosinophilic fasciitis - Oral: lp like, keratotic plaque - Genital: lp like, scarring - Nail: brittle nails, pterygium, - Systemic: eye, liver
H.P	VASCULAR INTERFACE DERMATITIS KCS necrosis	variable
DD	Viral Drug . TEN	LP, lichenoid drug reaction, AICD . SLE morphea system



	Acute gvhd	Chronic GVHD
Treatment	Topical , systemic steroid Tacrolimus Mfx , TNF alpha	Sun protection Phototherapy UVA Systemic steroid

ERYTHRODERMA



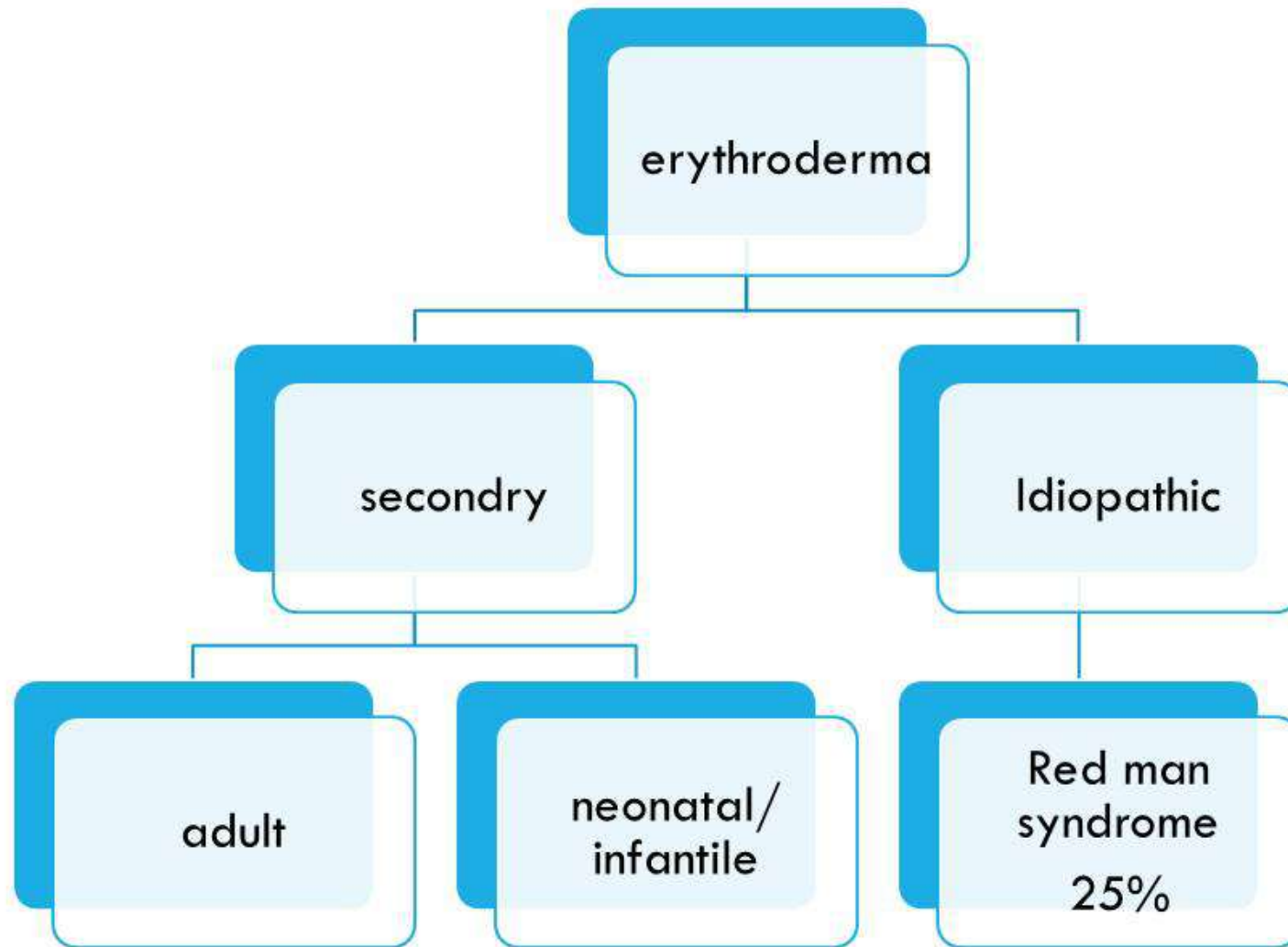
CONNECT ACADEMY - DERMA - BY DR. EMAN

ERYTHRODERMA

Erythema , scaling more than 90% of body

- Conteneu months to years
- Relapsing , remitting course
- Main cause of death is pneumonia
- Exacerbatng factors :uv radiation , drug





Adult erythroderma

Common
Ps
Atopic
dermatitis
Drug (aspgc) 😊

Less common
MF
PRP
Eczema
Para neoplastic
Pem. Follicle
Bullous
pemphigoid

Rare
Crusted
scabies
Congenital
ichthyosis
Lp
GVHD

Neonatal , infantile
erythroderma

➤ congenital
ichthyosis

1. CIE (bullous
non bullous
2. NETHORTON
syndrom

Immunodeficiency
✓ Wiskott Aldrich
syndrom

Inflammatory
Atopic
SD
PS

drug

Infection
Ssss
ccc

Other
PRP
GVHD
MASTOC
YTOSIS

Drug (ASPGC)

Allopuranol
Sulfasalazine
Sulfonamide
Phenytoin
B blocker
Gold
carbamazepine

DRUGS ASSOCIATED WITH ERYTHRODERMA

Common

- Allopurinol
- β -lactam antibiotics
- Carbamazepine/oxcarbazepine
- Gold
- Phenobarbital
- Phenytoins
- Sulfasalazine
- Sulfonamides*
- Zalcitabine

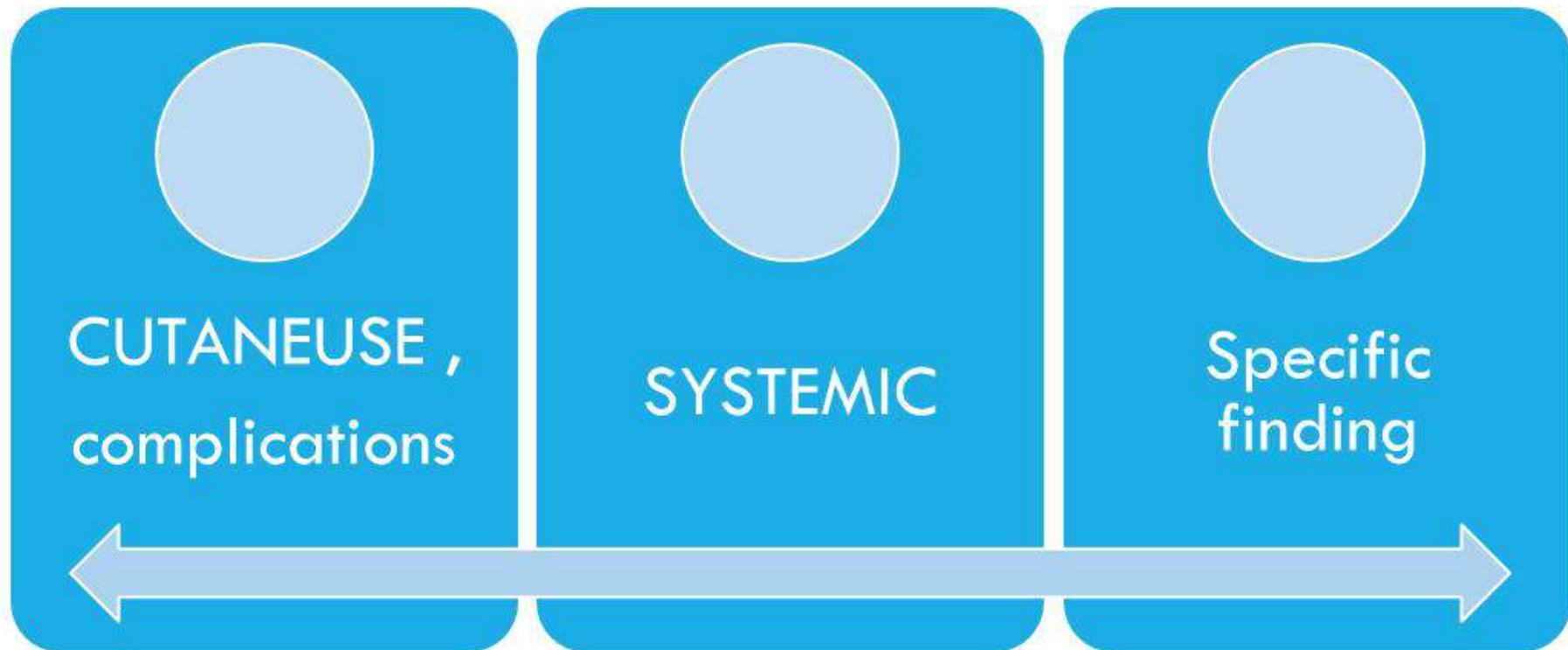
Less common

- Captopril/lisinopril
- Carboplatin/cisplatin
- Checkpoint inhibitors**
- Cytarabine
- Cytokines (IL-2/GM-CSF)
- Dapsone
- Diflunisal
- Fluindione
- Hydroxychloroquine/chloroquine
- Isoniazid
- Isotretinoin/acitretin
- Lithium
- Mercury compounds
- Minocycline
- Omeprazole/lansoprazole
- Ribavirin
- Telaprevir
- Thalidomide
- Tocilizumab
- Vancomycin†

Rare†

- Abacavir
- Amiodarone
- Aspirin (ASA)
- Aztreonam
- β -blockers
- Chlorpromazine
- Cimetidine
- Ciprofloxacin
- Clofazimine
- Mitomycin C
- Nifedipine/diltiazem
- Other NSAIDs
- Penicillamine
- Pentostatin
- Pseudoephedrine
- Rifampin
- St John's wort

CLINICAL FEATURES



CUTANEOUSE MANIFESTATION:



- erythema , scaling >90%
- Erythema become universal 12h -48h
- Scaling appear after 2-8
- Bright red hot dry skin
- PRURITIS >most common (increase in Sezary , dermatitis)

Complication :

1. Lichenifecation
2. Dyspigmentation
3. PPK in (prp, crusted scabies ,Sezary)
4. Nail changes: brittle , dullness , beaus line
5. Alopecia
6. Spk
7. Staph aureus

SYSTEMIC MANIFESTATIONS

1. Constitutional
2. General lymphadenopathy 50%
3. Peripheral oedema 50%
4. Hepatomegaly 20%
5. Splenomegaly
6. Ocular: ophthalmopathy, conjunctivitis

✓ 7 _INCREASE SKIN PERFUSION

1. Fluid loss
2. Electrolyte disturbance
3. Tachycardia
4. Hypothermia
5. High cardiac output heart failure
6. Cachxia , anaemia

SPECIFIC FINDING

CAUSES OF ERYTHRODERMA IN ADULTS			
Underlying disease	Clinical clues	Histologic clues	Additional hints
Common			
Psoriasis (Ch. 8)	• Pre-existing psoriatic plaques or lesions of pustular psoriasis • Often spares the central face • Nail changes (oil-drop, pits, onycholysis) • Subcorneal pustules • Inflammatory arthritis	• Confluent or focal parakeratosis • Epidermal hyperplasia with bottleneck-like rete ridges • Tortuous vessels in a slightly edematous papillary dermis • Neutrophils within epidermis; sparse lymphohistiocytic infiltrate within dermis • Often reduced or absent granular layer	• Personal or family history of psoriasis • Withdrawal of corticosteroids, methotrexate, cyclosporine or targeted immune modulators (biologic agents) • Receiving medication that can exacerbate psoriasis (e.g. lithium)
Atopic dermatitis (Ch. 12)	• Pre-existing lesions (flexures) • Severe pruritus • Lichenification, including eyelids • Prurigo nodularis	• Mild to moderate acanthosis • Variable spongiosis • Dermal eosinophils • Parakeratosis • Variable lymphohistiocytic infiltrate with exocytosis of lymphocytes • Variable dermal edema	• Elevated serum IgE, eosinophilia • Personal or family history of atopy (e.g. asthma, allergic rhinitis, atopic dermatitis) • Cataracts/keratoconus
Drug reactions (Ch. 21)	• Preceded by morbilliform or scarlatiniform exanthem • Facial edema • In dependent areas, may become purpuric	• Perivascular infiltrate with eosinophils • Lichenoid pattern • Lymphomatoid pattern • Spongiotic pattern • Apoptotic keratinocytes at all levels of epidermis • Vacuolar degeneration of basal layer • DRESS/DIHS – spongiotic and lichenoid patterns	• No history of skin diseases • More acute onset • Usually resolves within 2–6 weeks after withdrawal of responsible drug; possible exception is DRESS/DIHS • More common in HIV-infected patients
Idiopathic erythroderma	• Elderly men • Chronic, relapsing • Severe pruritus • Palmoplantar keratoderma • Dermatopathic lymphadenopathy	• Nonspecific • Acute forms – parakeratosis, spongiosis, and a moderate lymphohistiocytic infiltrate • Chronic forms – hyperkeratosis, psoriasiform acanthosis, and expanded papillary dermis	• Consider less commonly associated drugs (see Table 10.3) • Continue to re-evaluate for cutaneous T-cell lymphoma, including serial skin biopsies



Fig. 10.5 Idiopathic erythroderma. This is the type of patient who requires longitudinal evaluation to exclude the development of cutaneous T-cell lymphoma.





Fig. 10.4 Erythroderma due to atopic dermatitis. **A** Widespread erythema with fine white scaling and excoriations. **B** Fine white scaling and obvious pruritus in addition to circumoral pallor. *A, Courtesy, Lorenzo Cerroni, MD; B, Courtesy, Antonio Torrelo, MD*



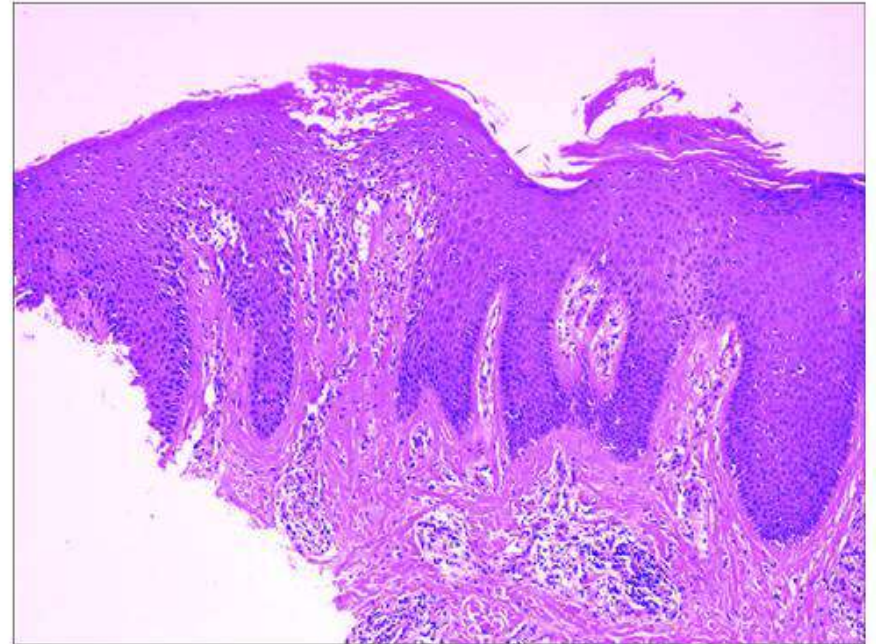
Fig. 10.6 Sézary syndrome. Diffuse erythroderma with a red-brown color. Note the evidence of scratching on the lower back. *Courtesy, Lorenzo Cerroni, MD.*



Fig. 10.9 Erythroderma due to pemphigus foliaceus. Generalized erythema with widespread scale-crusts and large areas of erosion.

HISTEOPATOLOGY

- ACUTE :SPONGIOSIS , PARAKERATOSIS
- Chronic :acanthosis , elongation of rete ridges
- Dilated B.V , odema in the skin



TREATMENT INITIAL MANGEMENT

1. Hospitalization
2. Stop any drug
3. Close monitoring
 1. Fluid intake
 2. body temperatue
 3. Electrolyte impalance

- Antibiotic :for 2ry infection
- Sedating antihistaminic
- Topical skin care
 - Tap water wet dressing
 - Greasy emollient
 - Low potency topical steroid

SPECIFIC TREATMENT

➤ ps :methotraxat

➤ Idiopathic , drug : systemic steroid



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